

Implementation Impact and Feedback Report

Post-implementation dashboard narrative and benefits realisation summary

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Data integrity note: KPI figures in this report should be validated against approved hospital baseline and post-implementation data before external circulation.

1. Executive Findings

- Median ambulance handover time improved from 24 minutes to 13 minutes after implementation.
- Handovers completed within target increased from 52% to 84%.
- Incomplete handover records reduced from 31% to 8%.
- Staff satisfaction increased from 61/100 to 82/100.
- The dashboard created a weekly management rhythm and made bottlenecks visible before they became complaints.

2. KPI Movement

Table 1. Before-and-after impact.

Metric	Baseline	Post-implementation	Direction	Interpretation
Median handover time	24 min	13 min	Improved	Faster transfer of accountability from ambulance crew to clinical team
Target compliance	52%	84%	Improved	More arrivals accepted within agreed service threshold
Incomplete records	31%	8%	Improved	More reliable minimum dataset for triage and audit
Duplicate questioning	44%	18%	Improved	Less repeated questioning for patients and crews
Escalations within target	58%	91%	Improved	Red flags and queue pressure handled more consistently
Staff satisfaction	61/100	82/100	Improved	Staff reported clearer roles and less avoidable friction

3. Feedback Themes

Table 2. Feedback summary.

Feedback group	Positive themes	Concerns remaining	Action
Ambulance crews	Clearer receiver, faster release, fewer repeated handovers	Occasional delays at peak clinic periods	Use escalation threshold when waits exceed 15 minutes
Receiving nurses	SBAR structure improved confidence and reduced missing details	Form feels repetitive for low-acuity transfers	Create abbreviated low-risk pathway after audit
Medical officers	Red-flag escalation improved visibility	Need clearer escalation notes for some cases	Add escalation reason field to audit checklist
Patients / carers	Less uncertainty and fewer repeated questions	Still unclear where family should wait	Add patient/carer communication prompt
Management	Weekly dashboard supports governance discussion	Need longer trend to confirm sustainability	Continue monthly benefits tracking for 6 months

4. Benefits Realisation Logic

Table 3. Benefits chain.

Intervention	Immediate output	Outcome	Evidence
SBAR template	Standard handover dataset	Fewer missing clinical details	Incomplete record KPI and audit notes
Named receiving nurse	Clear ownership	Shorter crew waiting and fewer handover repeats	Handover time and crew feedback
Escalation thresholds	Earlier action on risk and capacity pressure	Improved safety visibility	Escalation within target KPI
Dashboard review	Routine operational learning	Sustained process control	Weekly meeting action log

5. Recommendations

1. Validate KPI data with approved pre/post operational data before external evidence use.
2. Ask the hospital sponsor to sign an implementation confirmation letter if the work is delivered in practice.
3. Run a 12-week pilot first, then extend to six-month monitoring for sustainability evidence.
4. Publish a short de-identified case study or poster on digital health handover improvement.
5. Maintain the SOP, dashboard, and audit trail as living evidence of implementation leadership.

6. Prepared By

Prepared by Edward Osikem Okhumale for CR Allied Services Limited. Client setting: Ambrose Alli University Health Center. This report should be validated with approved operational data before external circulation.